

PATIENT REFERRAL DOCUMENT

LMU Klinikum Munich

Patient: Anna Becker

Date of Birth: 14.11.1978

Date: 15.05.2026

Referring Physician: Dr. Klaus Hartmann

Internal Medicine Practice, Maxvorstadt Munich

REASON FOR REFERRAL:

Patient presents with a 10-week history of persistent productive cough with episodes of blood-tinged sputum on three occasions over the past three weeks. Associated with progressive breathlessness on exertion and unexplained weight loss of 7kg over three months. Chest X-ray performed in practice shows a suspicious right upper lobe shadow. Non-smoker but significant occupational exposure history.

CLINICAL FINDINGS:

Blood pressure: 118/76 mmHg

Heart rate: 88 bpm

Temperature: 37.6 degrees Celsius

Oxygen saturation: 94% on room air · reduced

Respiratory rate: 20 breaths per minute

BMI: 21.2 · down from 24.1 three months ago

RESPIRATORY EXAMINATION:

Dullness to percussion right upper zone

Reduced breath sounds right upper lobe

No wheeze

No crackles

Mild use of accessory muscles on exertion

CURRENT MEDICATIONS:

No regular medications

Occasional paracetamol for pain

PAST MEDICAL HISTORY:

No previous respiratory conditions

Appendectomy 2005

No known allergies

INVESTIGATIONS PERFORMED:

Chest X-ray: 3.2cm opacity right upper lobe with irregular margins, possible satellite nodule identified. Report states · further investigation strongly recommended.

Full blood count: Mild normocytic anaemia · Hb 10.8 g/dL

CRP: 34 mg/L · mildly elevated

LFTs: Within normal limits

Sputum culture: No growth after 48 hours

OCCUPATIONAL HISTORY:

Worked in construction industry for 12 years with known asbestos exposure in older buildings. Wore mask inconsistently. Left construction work 2015.

Currently works as an office administrator.

FAMILY HISTORY:

No family history of lung cancer

Father · COPD, heavy smoker

SOCIAL HISTORY:

Non-smoker · never smoked

Alcohol · minimal, occasional wine at weekends

Lives with husband and two children

CLINICAL IMPRESSION:

Highly concerning presentation with right upper lobe opacity, haemoptysis, significant weight loss, and occupational asbestos exposure history.

Differential includes primary lung malignancy, mesothelioma, or infectious aetiology. Requires urgent CT chest with contrast, bronchoscopy, and respiratory specialist review. Anaemia may be related to underlying malignancy. Patient is anxious and needs prompt assessment and clear communication.

Urgency: Semi-urgent · please see within 48 hours.